

CLINICAL FINDINGS

The algorithm shown in Box 75-1 emphasizes the need to obtain a complete history, perform an adequate examination, and construct a differential diagnosis. Initiating investigations to exclude sexually transmitted disease or performing a biopsy may be indicated. Occasionally, a trial of treatment may help crystallize a diagnosis.

The responsibility for the elucidation of anogenital symptoms may need to be shared with the genitourinary physician, pediatrician, urologist, or colorectal surgeon.

History

Although classic dermatologic symptoms (itch, rash, lumps or bumps, or ulceration) may be present, the physician must be alert to symptoms of sexually transmitted disease (e.g., discharge, dysuria) and urologic disease (e.g., pain, hematuria, dysuria), as well as male sexual dysfunction.

Symptomatic male sexual dysfunction equates with male dyspareunia—that is, painful or difficult sexual intercourse. Many men will not volunteer such information, so specific inquiry should be tactfully made. The components of normal male sexual function are libido, erection, ejaculation, and orgasm.

Cutaneous Lesions

A systematic approach to the penis and prepuce is summarized in the algorithm (see Box 75-1). The foreskin should be gently retracted (if present), the gluteal and crural folds and the meatal lips parted, and the rectum examined digitally. Site, distribution, and morphology of lesions should be conventionally noted and analyzed.

Specific male genital signs include: - **Phimosis**: non-retractable foreskin; this condition should be regarded as a sinister situation because it impedes complete

inspection and palpation of the glans and coronal sulcus. Causes of phimosis are listed in Table 75-2. - **Paraphimosis**: foreskin fixed in retraction; it is rarer and can result from vigorous sexual activity, acute contact urticaria, acute allergic contact dermatitis, or lichen sclerosus (LSc). - **Balanitis**: inflammation of the glans penis. - **Posthitis**: inflammation of the prepuce. - **Dorsal perforation of the prepuce**: ulceration through the prepuce involving the glans and shaft, leaving the prepuce as an empty sleeve ventrally.

Balanitis and posthitis xerotica (obliterans) are terms sometimes used to describe the end stage of chronic balanitis and posthitis, characterized by scarring and atrophy.